

BIGLY LABS × DIS-CHEM

Future-State Scripting

A North Star for Dis-Chem

Defining the prescription experience we want to build — an *experience definition, not yet a blueprint*.

⚠ **Concept for review — not an official Dis-Chem artefact. Generated 2026-06-14 from the Bigly ground-truth vault.**

The North Star in one slide

So what: Price is regulated and fixed — so the durable advantage is the experience around the medicine: a calm, trusted, self-running scripting service for both South Africas.

WHERE WE'RE TAKING SCRIPTING

- ▶ The household medicine manager — one account, every family member, repeats that simply run.
- ▶ From a transactional, paper-and-queue errand to a managed care relationship.
- ▶ One service spine, two first-class funded paths — insured and cash.
- ▶ Automation does the toil; a named pharmacist stays visible and in control.

WHY IT MATTERS

- ▶ **65% negative** pharmacy sentiment — Dis-Chem wins on retail price but is losing on pharmacy trust.
- ▶ **Customers defect** to local pharmacies, willing to pay more for relationship care.
- ▶ **Chronic repeats** are the recurring relationship that compounds loyalty — the engine to own.
- ▶ **Buy-in first** — agree the experience here, then blueprint and build it.

WHY NOW — THE PROBLEM

Pharmacy pain is a trust problem, not an availability problem

So what: The medicine is usually there; what fails is the experience around it — and in healthcare, trust failures don't get a second chance.

THE PATIENT TRUST JOURNEY *(social listening, 1,270 coded comments)*



"I have been pulling at my hair trying to order my medication via the App or even using the WhatsApp service."

"Processed on my medical aid and out for delivery. I did not receive it." — real Dis-Chem customers, Jan-2025 complaints (n=360)

65% negative pharmacy sentiment

vs 65% POSITIVE on retail price/promo — the trust gap is specific to pharmacy

5 complaint clusters

chronic re-order (dominant) · delivery fails · accuracy · service/ownership · billing

Defection to local pharmacies

customers "willing to pay R460 more" for relationship-based care



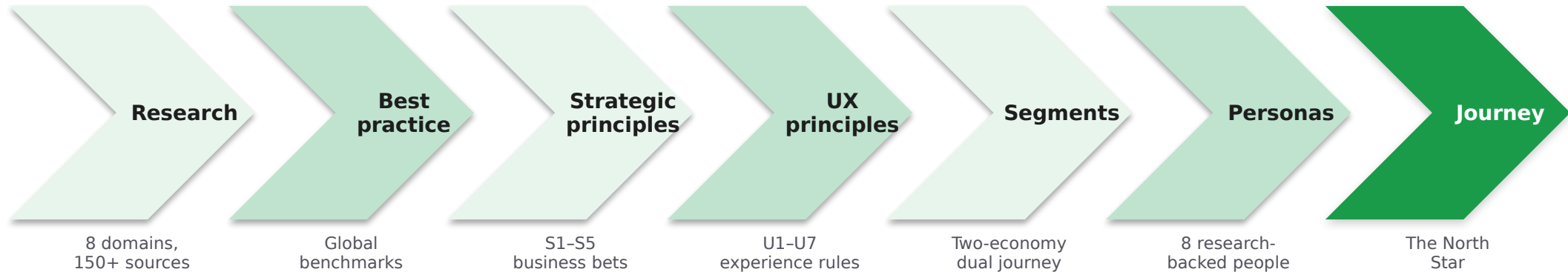
01

The method

How we built a defensible North Star — and the evidence it stands on.

How we built a defensible North Star

So what: A ground-truth research vault feeds a disciplined pipeline — research → best practice → principles → people → journey — so every design choice is traceable to evidence.



The ground-truth vault method. Every claim traces to a cited source; synthesis lives in a structured wiki; design outputs are defensible, not opinion.

This is the consulting discipline behind the North Star: we earn the outcome by trusting the method.

Scale and credibility behind the work

So what: ~150+ cited sources across 8 domains and A-D reliability tiers, anchored by Dis-Chem's own first-party primary research — this is grounded, not borrowed opinion.

8

research domains

regulatory, schemes, dispensing, digital, global, UX, consumer, Dis-Chem

150+

cited sources

one note per source: citation, claims, verbatim excerpts

A-D

reliability tiers

regulatory & clinical claims rest on A/B; signals labelled

50+

synthesis pages

cross-linked wiki — claims link back to their evidence

PLUS DIS-CHEM'S OWN PRIMARY RESEARCH

- ▶ **Complaints analysis** n=360 resolved (Jan 2025)
- ▶ **Social listening** 1,270 coded comments (Dec 2025)
- ▶ **Script-journey interviews** n=10 IDIs (Oct 2025)
- ▶ **Store-of-the-Future interviews** n=12 (Jan 2026)
- ▶ **Omnichannel tech roadmap** 24 capabilities (internal board)

What the world's leading pharmacies prove — and what transfers to SA

So what: Borrow mechanisms, not funding architectures: Australia, India and China transfer to South Africa far better than the US, UK or EU models.

PATTERNS TO BORROW

- ✓ **QR e-script token** — scanned at any pharmacy; printed/USSD/WhatsApp fallback (AU)
- ✓ **Nominated pharmacy / script list** — patient-consented, revocable (UK EPS, AU ASL)
- ✓ **Auto-refill / med-sync** — the refill is automatic; cancelling is the exception
- ✓ **Teleconsult → Rx → deliver** — one integrated loop, scoped to follow-up/chronic (IN, CN)
- ✓ **Adherence packaging + app wallet** — dose-by-time packs; carer mode (PillPack, MedAdvisor)
- ✓ **Price legible at point of commit** — SEP + capped fee already make this natural in SA

TRANSFER STRENGTH TO SA

Australia	Highest
India	High
China	High (loop + guardrails)
UK	Medium
EU / Germany	Medium (as a warning)
USA	Lowest (features only)

Reject: the US PBM layer, a single online-only rail, PIN/smart-card redemption, density-dependent speed promises.



02

Principles & people

The strategic bets, the experience constitution, the segments and the personas the journey is grounded in.

S1-S5 — the business bets

So what: Five strategic bets define what business this is: compete on service (not regulated price), own the chronic flywheel, and serve both economies on one spine.

S1

Serve both economies on one spine

Insured (~16%) and cash (~84%) are different journeys sharing one spine; the cash majority is never a degraded afterthought.

S2

Pharmacy as the health front door

Scripting is the entry to an ongoing care relationship, not a one-off transaction; the store + clinic estate is a strategic asset.

S3

Compete on service, not price

SEP + capped dispensing fee fix the price; the durable differentiator is the experience around the drug.

S4

Chronic-medicine flywheel = retention engine

Proactive, managed chronic repeats are the recurring relationship that compounds loyalty and adherence.

S5

Pharmacist at top-of-licence; automate the toil

Automation, central-fill and telepharmacy absorb the mechanics so pharmacists do the clinical, trust-building work.

U1-U7 — the experience constitution

So what: Seven UX rules govern how the experience must behave at every touchpoint — WhatsApp-first, never leave them guessing, money legible, graceful failure, dignity, fewer steps, adherence by design.

U1

**Channel ladder:
WhatsApp-first &
data-frugal**

WhatsApp/USSD/SMS first, app as enhancement; low-data, async, resumable.

U2

**Never leave them
guessing**

Honest real-time status at every step; the silent wait is the primary pain.

U3

**Make money
legible, early,
always**

Price + cover + co-pay shown before the till; cash/EFT/wallet; no cost shock.

U4

**Design for graceful
failure**

Plain-language recovery, never a dead end, always a route to a pharmacist.

U5

**Dignity, privacy &
language by default**

Stigma-aware, discreet, multilingual; caregivers/proxies are first-class users.

U6

**Reduce the journey,
don't just digitise it**

Eliminate steps and re-keying; one profile across channels; measure friction removed.

U7

**Adherence-as-
design**

Design the schedule, label and refill loop as a behaviour-change surface.

One spine, two first-class funded paths

So what: Funding — not age, device or geography — is the single most load-bearing design variable; the cash-paying majority must never feel like a degraded afterthought.

16%

84%

Insured (medical scheme) Cash / uninsured majority

INSURED JOURNEY — *Nomvula*

- ▶ Fund: real-time claim pre-adjudication; co-pay shown before commit
- ▶ Fulfilment: home delivery or collection by default
- ▶ Pay: scheme + saved method
- ▶ Steering: DSP / formulary / co-pay logic

CASH / UNINSURED JOURNEY — *Sipho*

- ▶ Fund: transparent cash price (SEP + dispensing fee) before commit
- ▶ Fulfilment: nearest collection / locker (delivery = premium)
- ▶ Pay: cash-on-collection + instant EFT / wallet, no saved card
- ▶ Steering: generic-first for affordability; CCMDD precedent

Eight research-backed people — three become the protagonists

So what: The journey is designed for real, evidenced people spanning the two-economy split — spotlighting an insured chronic patient, a cash WhatsApp-first customer, and a caregiver.



Nomvula Khumalo

Insured chronic, multimorbid

★ PROTAGONIST

54, Discovery, HTN + type-2 diabetes on the CDL. Wants several repeats to flow automatically, fully funded, no co-pay surprise.



Sipho Ndlovu

Cash, low-data, WhatsApp-first

★ PROTAGONIST

45, peri-urban, cheap Android, no street address. WhatsApp + SMS PIN + a collection point near home are the only things that reliably work.



Lerato Dlamini

Caregiver / proxy collector

★ PROTAGONIST

38, sandwich-generation. Runs medicine admin for her diabetic mother, two children and herself — multiple profiles, cycles and wallets.

THE FULL SET ALSO SPANS:

Thabo — cash-pay acute

Trudie — elderly poly-pharmacy

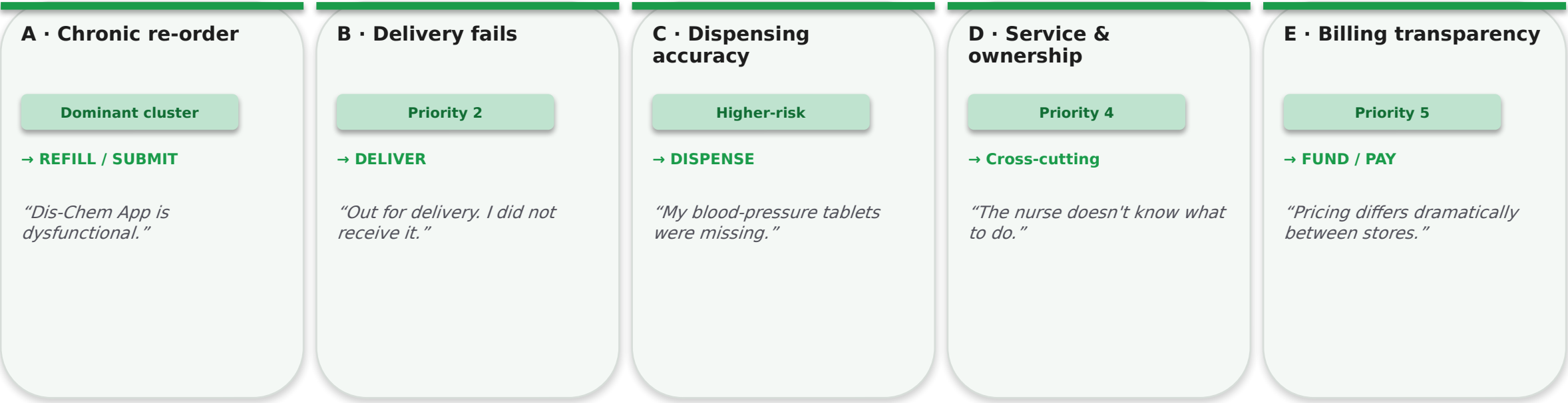
Aisha — time-poor professional

Mandla — public→retail bridge

Naledi — frontline pharmacist (staff)

Today's complaint clusters map cleanly onto the journey spine

So what: Most root causes sit in the digital and fulfilment seams — yet surface as 'pharmacy' complaints; fixing the counter alone won't fix them. This sets up the journey as the answer.



03

The North Star

The future-state experience — the household medicine manager, revealed.



The household medicine manager — a self-running repeat

So what: One person manages her own, her mother's and her child's medication in one place; the system predicts, dispenses and delivers — she barely has to think about it.

THE MOMENT ARC

- “My repeat is due — and I barely have to think about it”
- “I handle Mum's and my little one's meds in one place”
- “I know the cost and cover before I commit”
- “A named pharmacist has my back”
- “It's picked, checked and packed — and I watch it happen”
- “It arrives when they said — never left guessing”
- “When life throws a curveball, the system bends”

Source: [deliverables/05-tobe-future-state-journey.html](#) · [src-vn-cape-point-11](#)
Concept for review — not an official Dis-Chem artefact · Bigly Labs x Dis-Chem

Dis-Chem +Script Re-imaginedFUTURE-STATE CUSTOMER JOURNEY MAPbigly labs

⚠ Concept for review — not an official Dis-Chem artefact generated 2026-06-14 from the Bigly ground-truth vault · [deliverables/05-tobe-future-state-journey.html](#)

Script Re-imagined — the household repeat

A future-state customer journey for the household medicine manager — the self-running repeat. One person, one account, managing her own multimorbid chronic meds, her elderly mother's chronic meds and her child's occasional acute needs. Toggle the economy to see the same journey for an insured and a cash/uninsured household; open any moment for the story, today's baseline pain, and the capabilities behind the line.

Complex use case: The household medicine manager — the self-running repeat (a patient who is also a caregiver: own chronic meds + an elderly parent's chronic meds + a child's acute curveball, across one account).

VIEW

JourneyMy medicine cabinet

Speed

Convenience

Trust

Our philosophy: Trust > Convenience > Speed
Predictability beats speed — confidence comes from clarity and fallback, not from automation alone.
Customer-first surface; the ops & systems depth lives behind the line of visibility, on demand.

ECONOMY

Insured — Nomvula

Cash / uninsured — Siph

Expand all

Collapse all

PATIENT · HOUSEHOLD MANAGER

Nomvula

54 · Midrand · HR manager · Discovery member, HTN + type-2 diabetes on the CDL · app-capable, time-pressured · also manages her mother Gogo's chronic meds and her 7-year-old's acute needs from one account.

CURRENT FRICTION

The journey — click a moment to open it

MOMENT 1

My repeat is due — and I barely have to think about it

★●●

MOMENT 2

I handle Mum's and my little one's meds in the same place

●●●

MOMENT 3

I know the cost and the cover before I commit

★●●

MOMENT 4

A named pharmacist has my back

★●●●

MOMENT 5

It's picked, checked and packed — and I watch it happen

★●●●

1

MOMENT 1

My repeat is due — and I barely have to think about it

REFILL/RENEW → SUBMIT S4 U6 U7 U2

Confident · 5/5

15

Calm on the surface, depth on demand — and equal on both paths

So what: Each moment opens to reveal the experience behind the line of visibility; the dual-economy toggle proves the cash path is first-class, not a downgrade.

Trust

Convenience

Speed

Our philosophy: **Trust** > Convenience > Speed

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PATIENT - HOUSEHOLD MANAGER

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54 - Midrand - HR manager - Discovery member, HTN + type-2 diabetes on the CDL - app-capable, time-pressured - also manages her mother Gogo's chronic meds and her 7-year-old's acute needs from one account.

CURRENT FRICTION

Co-pay surprise at the till

Refill is opt-in and breaks at expiry

No household / proxy model

App / WhatsApp re-order dead-ends

PATIENT NEED

Keep three people's medicine flowing — automatically, fully funded, with no co-pay surprise and no admin — from one phone she

The journey — click a moment to open it

MOMENT 1

My repeat is due — and I barely have to think about it

★☆☆

MOMENT 2

I handle Mum's and my little one's meds in the same place

★☆☆

MOMENT 3

I know the cost and the cover before I commit

★☆☆

MOMENT 4

A named pharmacist has my back

★☆☆

MOMENT 5

It's picked, checked and packed — an can watch it happen

★☆☆

1 MOMENT 1

My repeat is due — and I barely have to think about it

REFILL/RENEW - SUBMIT S4 U6 U7 U2

Confident - 5/5

Story

Behind the line

PATIENT NEED

Keep the household's chronic meds flowing without remembering or re-doing admin.

My repeat is due — and I barely have to think about it

A WhatsApp message lands a week before run-out: "Hi Nomvula — your BP & diabetes meds and Gogo's repeat are due. Same as last month? ☒ Yes / ☐ Change / ☐ Snooze." One tap and it's handled. She didn't have to remember — the system did, for the whole household.

ACTIVITIES

Receives a proactive, low-data 'due soon' nudge covering every household member

Confirms the whole household's repeat in one tap (or edits a line)

Nothing to re-type, no app gate, no cold restart

Show today's baseline (the pain this answers)

Cited: pharmacy-complaints-pain-points - chronic-medicine-flywheel - caregiver-proxy-collector - dischem-scripting-tech-roadmap - adherence-tech-and-digital-therapeutics

A moment expanded — story + behind-the-line (insured)

Trust

Convenience

Speed

Our philosophy: **Trust** > Convenience > Speed

Predictability beats speed — confidence comes from clarity and fallback, not from automation alone.

Customer-first surface; the ops & systems depth lives behind the line of visibility, on demand.

ECONOMY

Insured — Nomvula

Cash / uninsured — Siphso

Expand all

Collapse all

PATIENT - HOUSEHOLD MANAGER

Siphso

45 - peri-urban Durban - informal income - hypertensive on a cheap Android, intermittent signal, no street address - also collects for his elderly mother (chronic, CCMD) and a young child's acute needs — cash, at the margin, near home.

CURRENT FRICTION

App-first flows lock him out

No price certainty before he commits

Delivery assumes an address he doesn't have

Two economies barely connect

PATIENT NEED

Manage the family's medicine with whatever phone and airtime he has

The journey — click a moment to open it

MOMENT 1

My repeat is due — and I barely have to think about it

★☆☆

MOMENT 2

I handle Mum's and my little one's meds in the same place

★☆☆

MOMENT 3

I know the cost and the cover before I commit

★☆☆

MOMENT 4

A named pharmacist has my back

★☆☆

MOMENT 5

It's picked, checked and packed — an can watch it happen

★☆☆

1 MOMENT 1

My repeat is due — and I barely have to think about it

REFILL/RENEW - SUBMIT S4 U6 U7 U2

Reassured - 4/5

Story

Behind the line

PATIENT NEED

Keep the household's chronic meds flowing without remembering or re-doing admin.

My repeat is due — and I barely have to think about it

An SMS / WhatsApp line he can read on any phone: "Sawubona Siphso — your BP tablets and your mother's repeat are due. Reply 1 to prepare for collection near home." No app, no data-heavy download — just a reply. The family's chronic supply stays ahead of the run-out.

ACTIVITIES

Gets a tiny, in-language 'due soon' SMS/WhatsApp prompt for his and his mother's chronic meds

Replies with one keypress (USSD works too)

Never has to install an app or load his data

Show today's baseline (the pain this answers)

Cited: pharmacy-complaints-pain-points - digital-access-and-whatsapp - public-sector-bridging-to-retail - dischem-scripting-tech-roadmap - courier-chronic-pharmacy-models

The same journey, cash / uninsured path (Siphso)

Low-friction & human: predictability beats speed (SOTF), the customer surface stays calm, and the ops/tech depth lives behind a line of visibility the reader can open. The cash majority gets the same dignity, transparency and fulfilment quality.

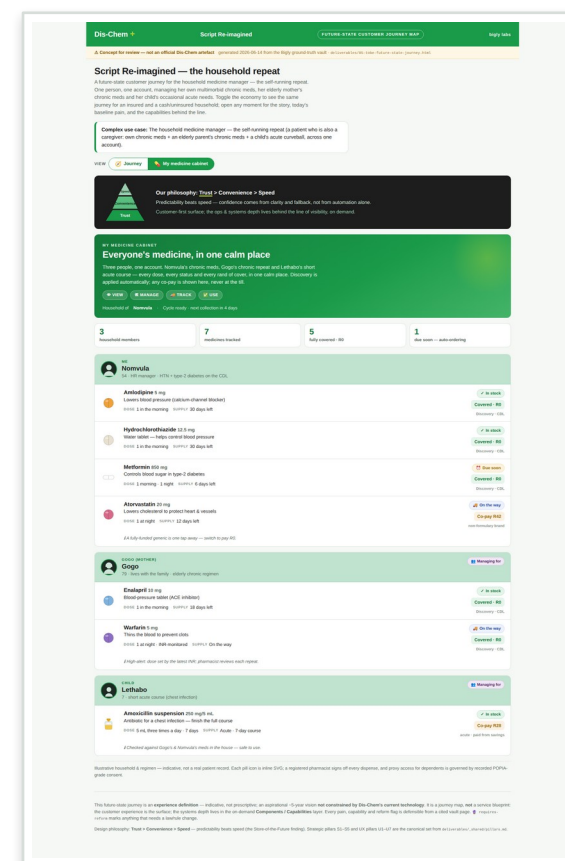
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“My medicine cabinet” — the hero moment

So what: Every household member's medication in one beautiful view — what to take, when, what's left and what's coming — turns admin into confidence.

WHY IT MATTERS

- ▶ **Every member, one view** — her own multimorbid regimen, her mother's chronic meds, her child's acute scripts — with a clear pill visual.
- ▶ **Predict, don't prompt** — the cabinet shows what's running low and pre-stages the repeat — the best refill is the one she didn't have to think about (U7, S4).
- ▶ **Adherence by design** — what to take and when, with hybrid digital + printed instructions and consented, opt-out-able reminders tied to routine.
- ▶ **Dignity & proxy by default** — caregivers are first-class users; POPIA-grade consent for managing a dependant's medication (U5).



★ The “manage my medications” showpiece moment

Source: [deliverables/05-tobe-future-state-journey.html](#) · [src-vn-cape-point-11](#)

Concept for review — not an official Dis-Chem artefact · Bigly Labs x Dis-Chem

The capability roadmap already exists — and we flag what needs reform

So what: ROWA, OCR, AI capture, BRIX, the E2E Dashboard and PICCUP are Dis-Chem's own roadmapped capabilities; the vision is buildable, with reform-dependencies named honestly.

ROADMAPPED CAPABILITIES (planned, not yet live)

ROWA	automated picker / dispensing robot — the proof-point for 'automate the toil' (S5)
OCR + AI capture	reads, standardises & routes the script; human fallback (PBQ)
BRIX	auto-dispense of eligible repeats + point-of-sale
E2E Dashboard	one workflow manager across dispensary, store, collection & delivery
PICCUP + VC	consolidation → driver → last-mile with proof-of-delivery
Virtual Manager	human SLA oversight of the virtual order queue

REQUIRES REFORM — flagged honestly

- ▶ Auto-dispense without human intervention vs pharmacist-oversight & valid-prescription rules
- ▶ Fully-paperless AES e-script acceptance (ECTA / Reg 33)
- ▶ Locker / unattended dispensing of scheduled medicines
- ▶ After-hours live scheme authorisation & 24/7 pharmacist oversight
- ▶ Remote pharmacist supervision / central-fill

Every future moment answers a real, quoted pain of today

So what: The gap is undeniable when you put today's customer words next to the future experience — the North Star is a direct response to the evidence.

TODAY (the quoted pain)	NORTH STAR (the moment that answers it)
<i>"Pulling my hair trying to order via the App or WhatsApp."</i>	➤ My repeat is predicted and one-tap — I barely have to think about it.
<i>"Out for delivery. I did not receive it."</i>	➤ Realistic windows, closed-loop tracking & proof-of-delivery — I'm never left guessing.
<i>"My blood-pressure tablets were missing."</i>	➤ ROWA picks, the pharmacist signs off, an itemised handover I can check.
<i>"The nurse doesn't know what to do."</i>	➤ A named pharmacist has my back; renewing is a tap, not a trek.
<i>"Pricing differs dramatically between stores."</i>	➤ Cost and cover are legible before I commit — no surprise at the till.

04

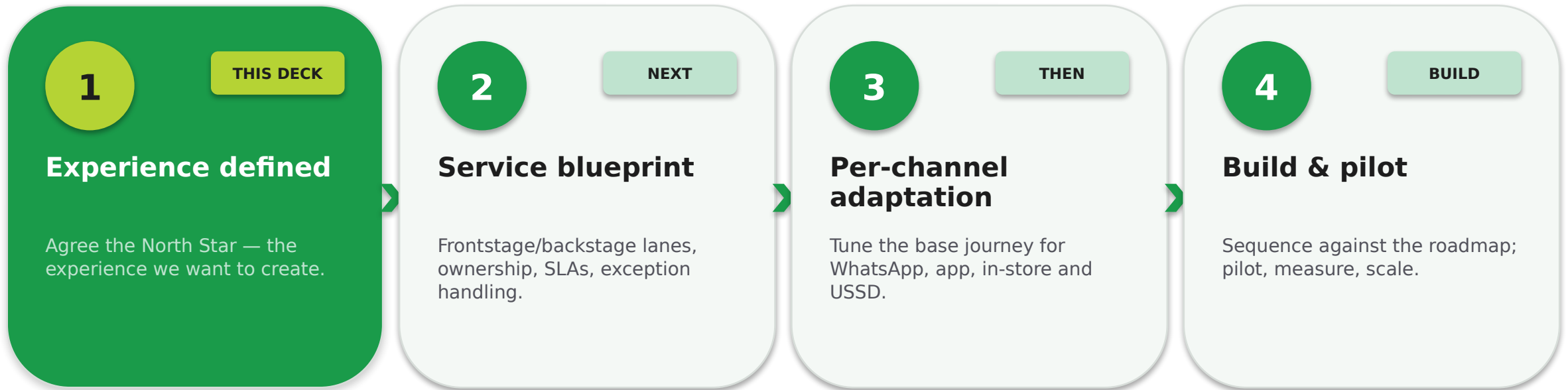
From here

What this deck is, what comes next, and the evidence behind it.



This deck is the groundwork — here is the road to build

So what: We've defined the experience and earned buy-in; the next steps turn it into a service blueprint, per-channel adaptations, and a build.



Methodology & source-reliability tiers

So what: How the vault works and how we weight evidence — the discipline behind every cited claim in this deck.

THE GROUND-TRUTH VAULT (3 layers)

Raw sources. one note per source: citation, key claims, verbatim excerpts — append-only evidence.

The wiki. synthesised, cross-linked entity/concept pages — claims link back to their sources.

The schema. conventions & workflows (ingest, query, lint) that keep it consistent and current.

Golden rule: *a wiki claim links to the source that backs it — synthesis, not narrative, in evidence.*

SOURCE RELIABILITY TIERS



Primary / authoritative

legislation, regulators (SAHPRA, SAPC, CMS, HPCSA), peer-reviewed, standards, company disclosures



Strong secondary

reputable analysts & consultancies, established trade press, gov-adjacent bodies



Useful but weaker

general news, vendor marketing, single-author blogs — verify load-bearing claims



Anecdotal / signal-only

forums, app-store reviews, social posts — never a factual basis; sentiment signal only

Full persona set & open questions / reform register

So what: The complete cast the design must satisfy, and the honest list of what we still need to verify or change in regulation.

THE 8 PERSONAS

1. **Nomvula Khumalo** — Insured chronic, multimorbid ★
2. **Thabo Mokoena** — Cash-pay acute customer
3. **Lerato Dlamini** — Caregiver / proxy collector ★
4. **Trudie van Wyk** — Elderly poly-pharmacy patient
5. **Sipho Ndlovu** — Low-data, peri-urban, WhatsApp-first ★
6. **Aisha Patel** — Time-poor urban professional
7. **Mandla Zwane** — Public-sector bridging to retail
8. **Naledi Mahlangu** — Frontline dispensary pharmacist (staff)

★ = *protagonist of the North Star journey*

OPEN QUESTIONS & REFORM REGISTER (selected)

- ▶ Real-time scheme pre-adjudication API (“can I claim?” dry-run) — exposed by any switch? Key enabler for U3.
- ▶ Legal boundary of auto-dispense for eligible repeats vs pharmacist oversight. 🧙
- ▶ SAPC acceptance of AES-signed e-scripts for S5/S6. 🧙
- ▶ Chain-of-custody for third-party last-mile couriers holding scheduled meds. 🧙
- ▶ 24/7 pharmacist oversight & live after-hours scheme authorisation. 🧙
- ▶ Exact dispensing-fee bands & CMS coverage split — re-verify vs primary gazette/PDF.

🧙 = *requires regulatory reform*. 48 reform flags & 63 unverified items tracked across the vault.